

Focus of Patient Education and Engagement

- Understanding obesity
- Consequences
- Future health concerns
- Rationale of why it is important to maintain a healthy weight and lifestyle
- Formal weight reduction program referrals
- Specific nutrition recommendations
- Resources/Links:
 - CDC, AHA, USPSTF Websites, Healthy People 2030
 - obesity medicine competencies: <https://obesitymedicine.org/wp-content/uploads/2020/09/Obesity-Medicine-Education-Collaborative-Competencies-Web.pdf>.

Resources

Brief Cutting Edge Report
CLINICAL TRIALS AND INVESTIGATIONS

Development of Obesity Competencies for Medical Education: A Report from the Obesity Medicine Education Collaborative

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Objective: Obesity Medicine Education Collaborative (OMEC) was formed to develop obesity-focused competencies and benchmarks that can be used by undergraduate and graduate medical education program directors. This article describes the developmental process used to create the competencies.

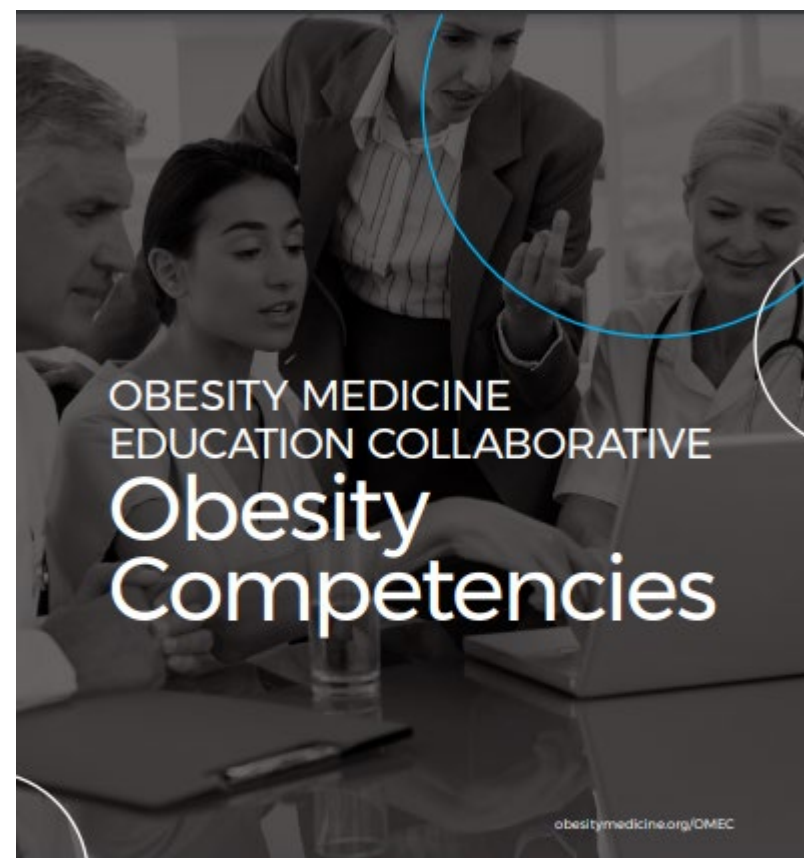
Methods: Fifteen professional organizations with an interest in obesity collaborated to form OMEC. Using the six Core Competencies of the Accreditation Council for Graduate Medical Education as domains and as a guiding framework, a total of 36 group members collaborated by in-person meetings, email exchange, and conference calls. An iterative process was used by each working subgroup to develop the competencies and assessment benchmarks. The initial work was subsequently externally reviewed by 19 professional organizations.

Results: Thirty-two competencies were developed across the six domains. Each competency contains five descriptive measurement benchmarks for evaluator rating.

Conclusions: This set of OMEC obesity-focused competencies is the first evaluation tool developed to be used within undergraduate and graduate medical training programs for both formative and summative assessments. Routine and more robust assessment is expected to increase the competence of health care providers to assess, prevent, and treat obesity. In addition to dissemination, the competencies and benchmarks will need to undergo evaluation for further validity and practicality.

Obesity (2019) **27**, 1063-1067. doi:10.1002/oby.22471

Obesity



Resources

Body Mass Index Screening:

Body Mass Index: Considerations for Practitioners (CDC):

<https://www.cdc.gov/obesity/downloads/bmiforpractitioners.pdf>

Talking With Patients About Weight Loss: Tips for Primary Care Providers (Niddk - NIH):

<https://www.niddk.nih.gov/health-information/professionals/clinical-tools-patient-management/weight-management/talking-adult-patients-tips-primary-care-clinicians>

3 Steps to Initiate Discussion About Weight Management With Your Patients (NIH):

<https://www.nhlbi.nih.gov/files/docs/resources/heart/steps.pdf>

Blood Pressure Control:

Team-Based Care to Improve Blood Pressure Control (CDC):

<https://www.cdc.gov/dhds/pubs/team-based-care.htm>

Counseling Patients On High Blood Pressure (Pharmacy Today):

[https://www.pharmacytoday.org/article/S1042-0991\(15\)31094-X/fulltext](https://www.pharmacytoday.org/article/S1042-0991(15)31094-X/fulltext)

Resources

Dietary Assessment and Counseling:

5 Tips for Better Patient Nutrition Counseling (AAPA):

<https://www.aapa.org/news-central/2020/11/5-tips-for-better-patient-nutrition-counseling/>

Top Tips to Counsel Your Patients on Diet, Exercise (Pharmacy Times): <https://www.pharmacytimes.com/view/top-tips-to-counsel-your-patients-on-diet-exercise>

Exercise Counseling:

Health Care Providers: Talk to Your Patients About Physical Activity (ODPHP)

https://health.gov/sites/default/files/2019-11/PAG_MYW_HCP_FS.pdf

Healthcare Providers' Action Guide (Exercise is Medicine):

https://www.exerciseismedicine.org/assets/page_documents/Complete%20HCP%20Action%20Guide.pdf

Assessment of readiness to change; behavioral counseling, Referral to a formal weight reduction program:

Assessing motivation and readiness to change for weight management and control: an in-depth evaluation of three sets of instruments (NIH):

https://www.ncbi.nlm.nih.gov/pmc/articles/PMC4426708/#_ffn_sectitle

Strategies to Increase Health System Referrals to

Type 2 Diabetes Prevention and Diabetes Management Programs (CDC):

https://www.cdc.gov/diabetes/pdfs/programs/stateandlocal/Referral_Strategies_Cleared_final_508.pdf

Diabetes Control:

Counseling Patients on Diabetes Self-Management (Pharmacy Times):

<https://www.pharmacytimes.com/view/counseling-patients-on-diabetes-self-management>

Primary Care: Diabetic Counseling for Newly Diagnosed Patients (Center for Adult Medicine & Preventative Care):

<https://campmedicine.org/blog/primary-care-diabetic-counseling-for-newly-diagnosed-patients/>

Health Care Providers (CDC):

<https://www.cdc.gov/diabetes/professional-info/health-care-providers.html>

Screening for Clinical Depression and Anxiety:

CDC Promotes Public Health Approach To Address Depression among Older Adults (CDC):

https://www.cdc.gov/aging/pdf/cib_mental_health.pdf

Strategies to Improve the Management of Depression in Primary Care (NIH - Jürgen Unützer, MD, MPH, MA and Mijung Park, PhD):

<https://www.ncbi.nlm.nih.gov/pmc/articles/PMC4127627/>

Healthy People 2030

Overweight and Obesity

Overview and Objectives

Evidence-Based Resources

Goal: Reduce overweight and obesity by helping people eat healthy and get physical activity.



About 2 in 5 adults and 1 in 5 children and adolescents in the United States have obesity,¹ and many others are overweight. Healthy People 2030 focuses on helping people eat healthy and get enough physical activity to reach and maintain a healthy weight.

Obesity is linked to many serious health problems, including type 2 diabetes, heart disease, stroke, and some types of cancer.² Some racial/ethnic groups are more likely to have obesity, which increases their risk of chronic diseases.³

Objective Status

0 Target met or exceeded

0 Improving

1 Little or no detectable change

2 Getting worse

3 Baseline only

- [Obesity Prevention and Control: Meal or Fruit and Vegetable Snack Interventions Combined with Physical Activity Interventions in Schools](#)
- [Abnormal Blood Glucose and Type 2 Diabetes Mellitus: Screening](#)
- [Obesity: Worksite Programs](#)
- [Obesity: Technology-Supported Multicomponent Coaching or Counseling Interventions – To Reduce Weight](#)
- [Obesity: Technology-Supported Multicomponent Coaching or Counseling Interventions – To Maintain Weight Loss](#)
- [Obesity: Multicomponent Interventions to Increase Availability of Healthier Foods and Beverages in Schools](#)
- [Obesity Prevention and Control: Digital Health Interventions for Adolescents with Overweight or Obesity](#)
- [Physical Activity: Interventions Including Activity Monitors for Adults with Overweight or Obesity](#)
- [Obesity: Meal or Fruit and Vegetable Snack Interventions to Increase Healthier Foods and Beverages Provided by Schools](#)
- [Weight Loss to Prevent Obesity-Related Morbidity and Mortality in Adults: Behavioral Interventions](#)

Depression and Anxiety Screening Tools

Patient Health Questionnaire-2 (PHQ-2)

PHQ-2 Scores and Proposed Treatment Actions

- Administer the full PHQ-9
- Conduct a clinical interview to assess for Major Depressive Disorder

1. Korenke K, Spitzer RL, Williams JB. The Patient Health Questionnaire-2: Validity of a Two-Item Depression Screener. *Med Care*. 2003, Nov;41(11):1284-92.
2. Kroenke K(1), Spitzer RL, Williams JB, Löwe B. The Patient Health Questionnaire Somatic, Anxiety, and Depressive Symptom Scales: a systematic review. *Gen Hosp Psychiatry*. 2010 Jul-Aug;32(4):345-59.

PHQ-9

Patient Health Questionnaire and General Anxiety Disorder (PHQ-9 and GAD-7)

Date _____ Patient Name: _____ Date of Birth: _____

Over the last 2 weeks, how often have you been bothered by any of the following problems?
Please circle your answers.

PHQ-9	Not at all	Several days	More than half the days	Nearly every day
1. Little interest or pleasure in doing things.	0	1	2	3
2. Feeling down, depressed, or hopeless.	0	1	2	3
3. Trouble falling or staying asleep, or sleeping too much.	0	1	2	3
4. Feeling tired or having little energy.	0	1	2	3
5. Poor appetite or overeating.	0	1	2	3
6. Feeling bad about yourself – or that you are a failure or have let yourself or your family down.	0	1	2	3
7. Trouble concentrating on things, such as reading the newspaper or watching television.	0	1	2	3
8. Moving or speaking so slowly that other people could have noticed. Or the opposite – being so fidgety or restless that you have been moving around a lot more than usual.	0	1	2	3
9. Thoughts that you would be better off dead, or of hurting yourself in some way.	0	1	2	3
Add the score for each column				

Total Score (add your column scores): _____

If you checked off any problems, how difficult have these made it for you to do your work, take care of things at home, or get along with other people? (Circle one)

Not difficult at all

Somewhat difficult

Very Difficult

Extremely Difficult

PHQ-9 Scoring

PHQ-9 score obtained by adding score for each question (total points)

Interpretation:

- Total scores of 5, 10, 15, and 20 represent cutpoints for mild, moderate, moderately severe and severe depression, respectively.
- Note: Question 9 is a single screening question on suicide risk. A patient who answers yes to question 9 needs further assessment for suicide risk by an individual who is competent to assess this risk.

Interpretation

Provisional Diagnosis and Proposed Treatment Actions		
PHQ-9 Score	Depression Severity	Proposed Treatment Actions
0 – 4	None-minimal	None
5 – 9	Mild	Watchful waiting; repeat PHQ-9 at follow-up
10 – 14	Moderate	Treatment plan, considering counseling, follow-up and/or pharmacotherapy
15 – 19	Moderately Severe	Active treatment with pharmacotherapy and/or psychotherapy
20 – 27	Severe	Immediate initiation of pharmacotherapy and, if severe impairment or poor response to therapy, expedited referral to a mental health specialist for psychotherapy and/or collaborative management

GAD-7

Over the last 2 weeks, how often have you been bothered by any of the following problems?
Please circle your answers.

GAD-7	Not at all sure	Several days	Over half the days	Nearly every day
1. Feeling nervous, anxious, or on edge.	0	1	2	3
2. Not being able to stop or control worrying.	0	1	2	3
3. Worrying too much about different things.	0	1	2	3
4. Trouble relaxing.	0	1	2	3
5. Being so restless that it's hard to sit still.	0	1	2	3
6. Becoming easily annoyed or irritable.	0	1	2	3
7. Feeling afraid as if something awful might happen.	0	1	2	3
Add the score for each column				

Total Score (add your column scores): _____

If you checked off any problems, how difficult have these made it for you to do your work, take care of things at home, or get along with other people? (Circle one)

Not difficult at all

Somewhat difficult

Very Difficult

Extremely Difficult

UHS Rev 4/2020

Developed by Drs. Robert L. Spitzer, Janet B.W. Williams, Kurt Kroenke and colleagues, with an educational grant from Pfizer Inc.
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Scoring GAD-7 Anxiety Severity

This is calculated by assigning scores of 0, 1, 2, and 3 to the response categories, respectively, of "not at all," "several days," "more than half the days," and "nearly every day."
GAD-7 total score for the seven items ranges from 0 to 21.

0–4: minimal anxiety

5–9: mild anxiety

10–14: moderate anxiety

15–21: severe anxiety